

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Global Data, 2023 · IHME / Our World in Data

Dattani, Rodés-Guirao, Ritchie & Roser (2023)



Hundreds of Millions Affected Worldwide

1 in 3

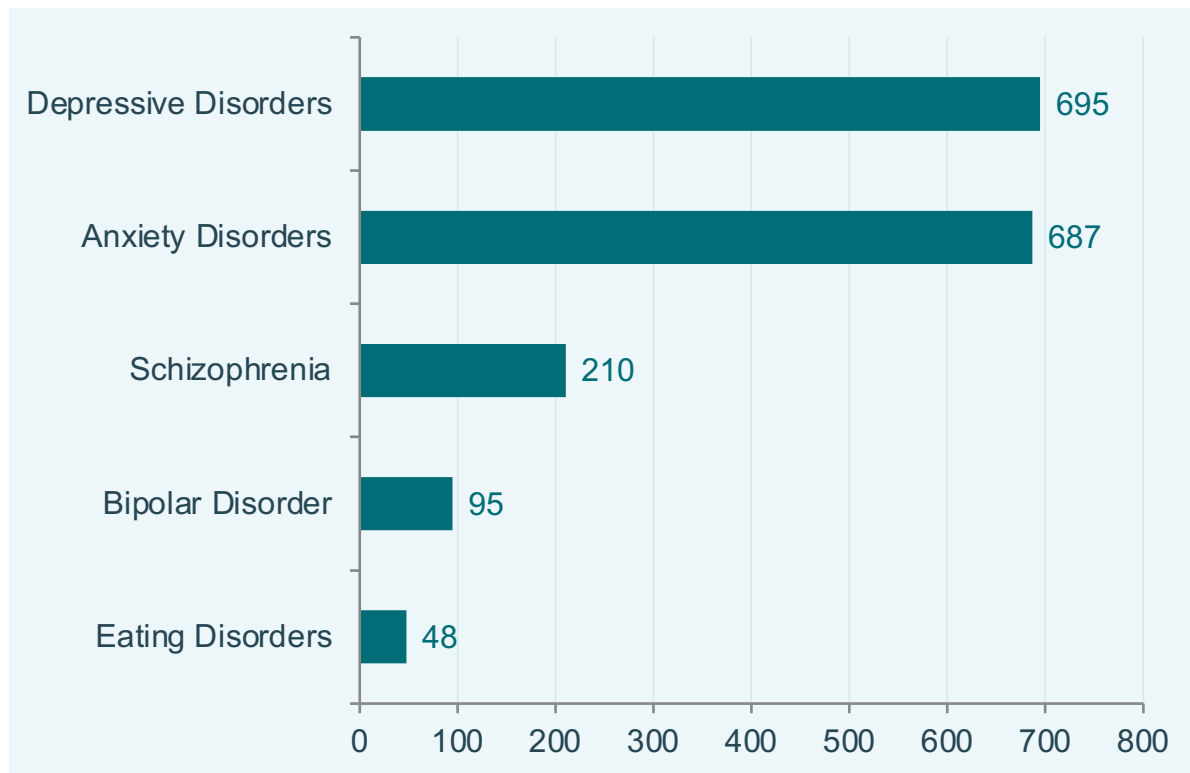
Women — Lifetime Depression Risk

1 in 5

Men — Lifetime Depression Risk

Mental health conditions range from common disorders (depression, anxiety) affecting hundreds of millions, to less prevalent but severely disabling conditions (schizophrenia, bipolar disorder). Understanding this spectrum is critical for effective resource allocation.

Depression and Anxiety Dominate the Global Mental Health Burden



Key Findings

Depression & anxiety each exceed 685 DALYs per 100K — nearly equal in magnitude

Together they account for ~80% of the total burden across these five categories

Schizophrenia (210.2) reflects severe disability despite lower prevalence

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide

Depressive Disorders

694.6

DALYs per 100,000



Anxiety Disorders

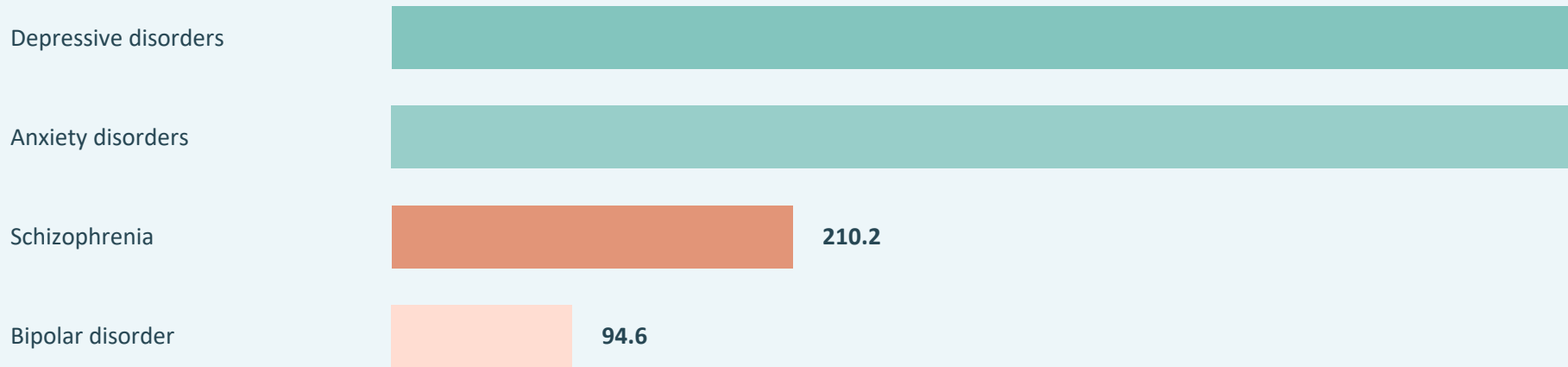
686.5

DALYs per 100,000



Together, depression and anxiety account for approximately 80% of mental health DALYs across major categories — dwarfing schizophrenia (210.2), bipolar disorder (94.6), and eating disorders (47.5).

Schizophrenia and Bipolar: Severe Individual Impact Despite Lower Prevalence



While depression and anxiety dominate population-level burden, schizophrenia and bipolar disorder carry a disproportionately high burden per affected individual. Specialized, sustained care for these severe conditions must be maintained alongside scaled-up common disorder interventions.

Treatment Is Available but Often Lacking or Poor Quality

Mental illnesses are treatable and their impact can be reduced. Yet treatment remains inadequate for millions — particularly in low- and middle-income countries.



S

Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or people they know.



A

Access

Treatment is often lacking, especially in low- and middle-income countries.



Q

Quality

Even where treatment exists, it may be of poor quality or not evidence-based.



U

Underreporting

Discomfort with disclosure means true prevalence is likely higher than reported figures suggest.

How People Cope: From Medication to Faith to Family

Global survey data reveals wide variation in how people with anxiety or depression respond to their condition — shaped by culture, economics, and healthcare access.



Rx

Prescribed Medication

Common in higher-income settings. Antidepressants and anxiolytics are the most widely used pharmacological interventions for depression and anxiety.



Religious or Spiritual Activities

Prevalent across many regions worldwide, particularly in Africa, Latin America, and parts of Asia. Faith communities often provide the first line of social support.



T

Talking to Friends or Family

A widely reported coping mechanism across income levels. Social support networks play a critical protective role and are central to many community-based mental health programs.

Policy note: Cultural diversity in coping strategies requires context-sensitive interventions — not a one-size-fits-all approach.

KEY TAKEAWAYS

Prioritizing Depression, Anxiety, and Closing the Treatment Gap

1

Depression and anxiety each generate ~690 DALYs per 100K — together representing approximately 80% of the mental health burden across major categories.

2

Schizophrenia (210.2 DALYs per 100K) carries a disproportionately high per-individual burden relative to its prevalence.

3

Lifetime depression risk is substantial: 1 in 3 women and 1 in 5 men will experience major depression.

4

A large treatment gap persists — stigma, poor access, and inadequate quality mean the true burden is likely underestimated.

5

Policy should scale evidence-based interventions for common disorders while maintaining specialized care for severe conditions.